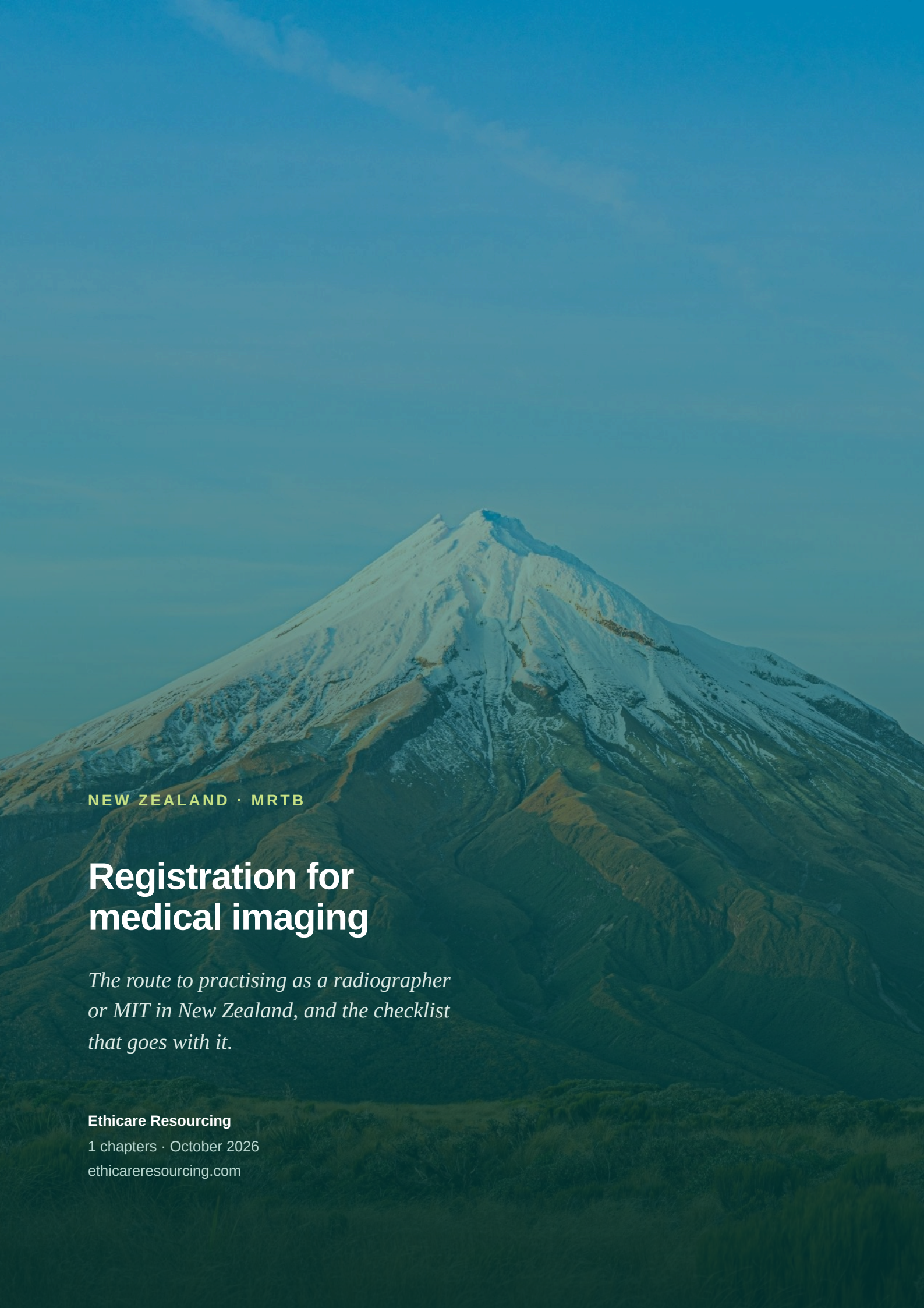




NEW ZEALAND · MRTB

Registration for medical imaging

*The route to practising as a radiographer
or MIT in New Zealand, and the checklist
that goes with it.*

Ethicare Resourcing

1 chapters · October 2026

ethicareresourcing.com

What's inside

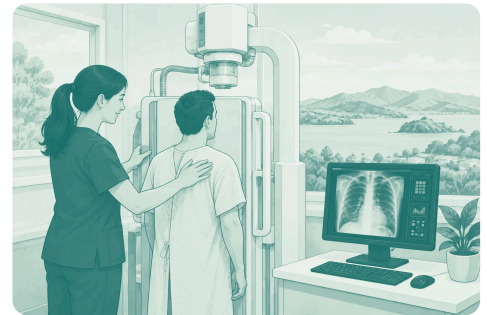
01 Registering with the MRTB

This guide is free, and so is everything in it. Every chapter here is also on the website, where it is kept up to date — so if you are reading this some months after downloading it, check the site for anything that looks time-sensitive. Fees, tax rates and immigration settings change without much notice.

We don't print these. A PDF costs nothing to send and nothing to ship, which felt like the right answer for a company that spends its days moving people around the world.

Registering with the MRTB

For internationally qualified medical imaging technologists, sonographers, nuclear medicine technologists and radiation therapists: how registration in New Zealand works, in the order you will meet it — with the checklist built into the page so nothing you tick goes out of date.



Regulator

Medical Radiation Technologists Board (MRTB), under the Health Practitioners Competence Assurance Act

Source

The Board's published requirements at mrtboard.org.nz. Where this page and the Board differ, the Board is right.

Status

Checked 3 October 2026 against mrtboard.org.nz — the scopes of practice notice, the qualification assessment pathway table and the online examination policy (v5, 4 June 2026). Fees and processing times are not printed here because the Board revises them.

Fees & timings

Deliberately not printed. The Board publishes both and changes them; check its current notice before you budget.

AT A GLANCE

Registration comes first

In New Zealand you generally need to be registered — or clearly registrable — before an employer progresses your application. Plan everything else on top of it.

You register into a scope

Medical imaging, MRI, sonography, nuclear medicine or radiation therapy. MRI and sonography are scopes in their own right; mammography is a specialisation inside the imaging scope. Your qualification decides the scope, not your job title.

Two questions, one decision

Are you suitably qualified, and are you currently competent and safe? Every document you supply answers one of those two.

Supervision is normal

Many international registrants start with a supervised period. It is a structured settling-in, not a doubt about your ability.

What's inside

Read it once to understand the shape of the journey, then come back to each section as you reach it. The checklist at the end remembers what you have ticked, on this device.

[Understanding the Board](#)

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[The checklist — 36 items, tickable](#)

SECTION 01

Understanding the Board

The Medical Radiation Technologists Board registers and oversees medical radiation professionals in New Zealand under the Health Practitioners Competence Assurance Act. Its job is to protect the public by making sure everyone practising in a regulated imaging or radiation therapy role is qualified, competent and fit to do so.

The titles and clinical roles are legally protected. You cannot practise, or call yourself a registered practitioner, without being registered with the Board and holding a current practising certificate. That is not a formality — it is the mechanism that tells patients, employers and colleagues you have met a national standard.

Scopes of practice

The Board registers you into a defined scope — medical imaging, MRI, sonography, nuclear medicine or radiation therapy — and you must practise within it. Your qualification and experience are assessed against the scope you apply for, which is why matching your application to the right scope from the start matters so much.

Registration and the Annual Practising Certificate

Registration confirms you have met the standard. The Annual Practising Certificate (APC) is what permits you to work, year to year. Think of registration as the licence and the APC as keeping it current: you must hold a valid APC while working, renewed annually alongside a declaration about your health, conduct and continuing professional development. The Board can ask for evidence of CPD, so keep records from day one.

Qualification → Registration → Annual Practising Certificate → Practise in New Zealand

SECTION 02

Which professions register with the MRTB

Your scope is determined by your qualification and the work you are trained and experienced to do — not by your job title, which varies from country to country.

Scope

Medical Imaging Technologist

Diagnostic images across general X-ray, CT, fluoroscopy and theatre imaging. Technical precision with direct patient care, often across many clinical settings.

Scope

Sonographer

The Board defines this scope by three modalities — general, vascular and cardiac ultrasound imaging. Highly autonomous and interpretive, needing strong clinical judgement.

Scope

Nuclear Medicine Technologist

Functional imaging with radiopharmaceuticals — PET and SPECT — blending imaging with the safe handling of radioactive materials.

Scope

Radiation Therapist

Planning and delivering radiation treatment, most often within oncology services, across a whole course of treatment.

Scope

Magnetic Resonance Imaging (MRI) Technologist

A gazetted scope of practice in its own right, not part of the imaging scope. There is no mutual-recognition shortcut into it: see Section 03.

Not a scope

Mammography

Mammography is a specialisation within the Medical Imaging Technologist scope, not a separate registration. You register as a Medical Imaging Technologist and practise into it with the appropriate training.

Not sure which scope applies to you?

Ask before you apply, not after. Assessment against the wrong scope costs months. [We can help you map your background to the right scope](#) so the application is judged against the correct standard from the start.

SECTION 03

Registration pathways and how you are assessed

At its heart, registration asks two questions: are you suitably qualified, and are you competent and safe to practise within the scope you are applying for? The pathway your application takes is simply the route the Board uses to answer those two questions for your background.

Qualification assessment. The Board first considers whether your qualification is comparable to the New Zealand standard for the scope. Some countries and institutions are well recognised; others need closer examination of course content and clinical hours.

Competence assessment. Beyond the paper, the Board wants assurance you are currently competent — recent practice, employment references and your clinical history.

Requests for more information. Common, and not a red flag: a clearer document, an extra reference, clarification of scope. Responding promptly and completely is the single biggest thing within your control.

POSSIBLE OUTCOMES

Being registered with a supervised period is normal, not a setback

Most internationally qualified applicants are registered with an initial period of supervised practice. It is a structured way of supporting you while you settle into the New Zealand system, equipment and protocols — not a sign the Board doubts your ability. Expect it rather than hope to avoid it: on the Board's own pathway table, a qualification from one of its six recognised countries *with* three or more years' practice in the five years before you apply still leads to registration with a condition of supervision. Less recent practice than that, and an examination comes first, with supervision on top.

The six recognised countries are Australia, Canada, Ireland, South Africa, the United Kingdom and the United States.

Registration granted — you meet the standard and proceed to your APC.

Registration with supervised practice — you can work, under the supervision of an experienced colleague for an initial period.

Conditions or further requirements — pending additional evidence, assessment or an examination.

Further assessment required — the Board asks for more before deciding.

If a supervised period applies, factor it in early: you will need an employer and a supervisor who can support it. Many employers are used to this and have arrangements ready.

The online examination pathway

For some applicants the assessment leads to an online examination. The Board's own policy sets the trigger: the examination applies to applicants who "do not hold a substantially equivalent qualification". In practice equivalence turns on two things together — where you trained, and how recent your practice is. A qualification from one of the six recognised countries backed by three or more years' practice in the last five avoids the examination; less recent practice than that, and it is required. It is a recognised, formal pathway, not a penalty, and the Board runs one for every one of its five scopes.

This is where scope matters, and it is the single most important thing on this page if you are an MRI technologist or a sonographer. Australian-registered practitioners in diagnostic radiography, nuclear medicine or radiation therapy are exempt from the examination under trans-Tasman mutual recognition. The Board states plainly that those provisions "do not apply to Australian-registered practitioners wanting to register in either the Sonographer or MRI Technologist scopes of practice in New Zealand". There is no mutual-recognition shortcut into either scope from anywhere. Every applicant goes through the full overseas-trained assessment, so whether you sit the examination comes down to where you trained and how recent your practice is — and for many people in these two scopes, that means sitting it. Anticipate it rather than be caught out.

The exam is sat online under remote proctoring: a supervisor verifies your identity (two forms of ID, one a government-issued photo ID, plus your authorisation code) and oversees the session. The fee is paid in advance and forfeited if you are refused access on the day for failing to establish your identity — have your documents in order.

1

Application received

The Board confirms your application and begins its review.

2

Qualification assessed

Checked against the New Zealand standard for your scope.

3

Competence and recent practice assessed

Your references and recent practice confirm your skills are current.

?

Further information, if needed

A routine step — a clearer document or extra detail.

✓

Registration decision

Confirmed, sometimes with conditions or a supervised period.

SECTION 04

Eligibility requirements

The Board's requirements look like a long checklist. They are easier to satisfy once you understand why each one exists — so that is how they are explained here.

Qualifications

Required: a recognised qualification appropriate to the scope you are applying for.

Why: your qualification is the foundation of safe practice. The New Zealand qualifications that lead to registration are bachelor's degrees (Level 7 on the New Zealand Qualifications Framework), so that is broadly the benchmark. The Board does not simply count years: it assesses whether your qualification is equivalent to, or as satisfactory as, an approved New Zealand qualification for your scope — content, clinical hours and outcomes, not level alone. If you are not certain how yours compares, check before you apply.

Recent practice

Required: evidence that you have practised recently and to a sufficient extent.

Why: skills fade when unused. If you have had a significant break, flag it early so it is addressed rather than discovered mid-assessment.

English language

Required: evidence of English language ability — Section 07.

Why: clear communication is a patient-safety issue: referrals, explaining procedures, precise communication with patients and colleagues.

Fitness to practise

Required: a declaration, and sometimes evidence, about your physical and mental health as it relates to safe practice.

Why: this is about practising safely, not about excluding people with health conditions. The vast majority of declarations raise no issues.

Professional conduct

Required: disclosure of any disciplinary history, conditions or investigations.

Why: the Board must protect patients. Full, honest disclosure — even of matters you think minor or resolved — is always right. Non-disclosure is treated far more seriously than the underlying matter usually is.

Certificates of Good Standing

Required: one from every regulator you have been registered with, sent directly to the MRTB.

Why: they are how one regulator tells another you have no unresolved concerns. They have a limited validity window, which makes their timing one of the trickier parts of an application — Section 05.

Read every requirement as a question.

The Board is asking whether you are safe to practise. Frame each document as the answer to that question and your application becomes far more persuasive.

SECTION 05

Preparing your application

A strong application is rarely about doing anything clever. It is about assembling clean, complete, correctly certified documents and presenting them well. The Board describes these documents as a legal requirement and says provision of them is non-negotiable, so treat the list as a list rather than

a guide.

How it looks matters more than people expect. The Board is forming a picture of you as a professional, and a folder of crooked, half-legible photocopies quietly undermines that picture even when every document is genuine. Clean colour scans, pages straight and square, nothing cut off, consistent file names, certification and translations done properly the first time. Treat it like a portfolio you would be proud to hand over.

Passport

Why

Confirms identity and nationality; underpins your visa and right to work.

Where

Your existing passport — valid, and not expiring mid-process.

Mistake

A passport close to expiry, or a scan that crops the machine-readable strip.

Tip

Scan the full photo page flat, in colour, all four edges visible.

Qualification certificates

Why

They prove you hold the qualification registration is based on.

Where

Your awarding university or college; request certified copies early if originals are missing.

Mistake

Only the final certificate, when the Board also wants the detail of what you studied.

Tip

Keep certificates and transcripts together — they are assessed as a pair.

Academic transcripts

Why

Course content, clinical hours and modules — often what lets the Board map your training to a scope.

Where

Your institution's registry; many issue official transcripts directly.

Mistake

Missing pages, or transcripts that do not show clinical placement hours.

Tip

If the transcript is light on clinical detail, add a course outline or syllabus.

Employment history and CV

Why

Demonstrates recent practice and the breadth of your experience.

Where

Compiled by you — accurate and complete.

Mistake

Unexplained gaps, or dates that do not match the rest of the application.

Tip

Account for every period. A one-line explanation of a break beats an unexplained hole. [Build your CV](#) is written for this.

Certificates of Good Standing

Why

Confirm good standing with every regulator you have registered with.

Where

Requested from each regulator and sent directly to the MRTB — not via you.

Mistake

Requested too early so they expire before assessment; or a regulator from years ago forgotten.

Tip

Limited validity, so timing is everything. Line them up to arrive when the application is ready, not months before.

Employment references

Why

They verify competence and recent practice — so important they have Section 06 to themselves.

Where

Current and recent supervisors or managers.

Mistake

A referee chosen for seniority rather than knowledge of your clinical work.

Tip

Brief your referees properly — read Section 06 before approaching anyone.

Name change documents

Why

If your name differs across documents, the Board needs to link them to one person.

Where

Marriage certificate, deed poll or equivalent.

Mistake

A name difference between a degree certificate and a passport, overlooked.

Tip

Check every document carries a consistent name, and include the bridging document if not.

Two certificates of good character

Why

A separate requirement from good standing, and the one applicants most often miss entirely. The Board asks for two — one personal, one professional.

Where

A professional colleague and someone who knows you personally. Neither is the same thing as a clinical reference or a certificate of good standing.

Mistake

Assuming your Certificates of Good Standing cover this. They do not.

Clinical logbook

Why

Your record of cases and procedures from training. It evidences the clinical content behind your qualification.

Where

Your training institution or your own records. Often left behind in a move — look for it before you need it.

Mistake

Finding out it is in a box in another country, six weeks into the application.

Full course curriculum, in English

Why

It lets the Board map what you were actually taught against the New Zealand standard for your scope.

Required, not a nice-to-have.

Where

Your institution's registry or course handbook for your year of study — not the current version of the course.

Mistake

Sending the modern prospectus instead of the curriculum you were taught.

Thesis or dissertation

Why

Where your qualification included one, the Board lists it among the documents it asks for.

Where

Your institution, or your own copy.

Mistake

Omitting it because it feels academic rather than clinical.

Clinical experience form

Why

The Board's own form, capturing your practice in the shape it wants to assess.

Where

Downloaded from the Board, completed from your records.

Mistake

Filling it from memory so it quietly disagrees with your CV and employment certificates.

Certificates of employment

Why

One from every employer you have had in the last five years, confirming the work you have claimed.

Where

Each employer's HR department. Start with the oldest — former employers are the slowest part of most applications.

Mistake

A single summary letter in place of individual certificates.

Evidence of two years' overseas employment

Why

Where it applies to you, the Board wants start and end dates, contracted weekly hours and job title.

Where

Your employers — ask for it in exactly that form.

Mistake

A letter confirming you worked there, with none of the three things the Board asked for.

Certified copies and translations

Why

Certification confirms a true copy; translation makes non-English documents assessable.

Where

An approved person such as a notary; a recognised, independent translator.

Mistake

Wrong or incomplete certification wording; translations not independently certified.

Tip

Confirm exactly who may certify and the wording required before you spend money. Getting it wrong means doing it twice.

One folder, one file per document.

When everything is gathered, certified and in date at the same time, the application itself takes a fraction of the effort.

SECTION 06

References are the part most within your control

Your qualification proves what you were trained to do. Your references prove what you do, now, in clinical practice. The Board relies on them to confirm you practise safely and competently. A vague reference leaves a gap the Board may have to fill by asking for more — which means delay.

Choosing the right referee

The instinct is to ask the most senior person available. Resist it. The best referee has directly supervised or worked alongside your clinical practice recently and can speak to it in specific terms. A clinical lead who knows your work beats a department head who barely does.

A poor reference

“I have known Sam for some years. Sam is hard-working and a pleasure to work with. I am happy to recommend Sam.”

No clinical detail, no dates, no scope, no basis for assessing competence.

A strong reference

“As Imaging Lead, I have directly supervised Sam for three years. Sam independently performs general X-ray, CT and MRI to a high standard, including complex and emergency cases, and consistently demonstrates safe, patient-centred practice.”
Specific, dated, scope-clear, credible.

What a reference that never prompts a follow-up contains

The referee's full name, job title and professional relationship to you

How long, and in what capacity, they have known your work

The specific scope and modalities you work within — settings and responsibilities

An assessment of your clinical competence, safety and professionalism, with examples

Confirmation of the dates and the hours or extent of the practice described

Official letterhead, signature, date and verifiable contact details

Brief your referee

Most weak references are weak because the referee was not told what was needed. Explain that this is for professional registration in New Zealand, share which scope you are applying for, and tell them what the Board values: clinical detail, dates, scope and a clear assessment of competence. Ask early — a referee on leave when you need them is a common, avoidable delay.

We can review references before submission against what the Board looks for and flag anything likely to prompt a follow-up. The content always remains your referee's own honest, independent assessment.

SECTION 07

English language requirements

This exists for one reason: patient safety. Clear communication in a clinical setting is non-negotiable, so the Board needs objective assurance.

Test

IELTS (Academic)

Widely accepted. The Board sets minimums overall and in each component — listening, reading, writing, speaking. A strong overall score does not help if one component falls short, so prepare evenly.

Test

OET

The healthcare-specific alternative many clinicians prefer because its tasks are set in medical contexts. Minimum grades apply in each of the four components.

Alternative evidence

English-medium training and practice

In some circumstances the Board accepts alternative evidence in place of a test — for example an English-medium education and career, or employer references. Eligibility is specific and decided by the Board; check before assuming you qualify.

Results expire.

Typically around two years from the test date. Sitting the test early and then taking many months to assemble the rest of the application is a frequent reason for paying to sit it again. Time it to remain valid through to assessment. And even fluent speakers stumble on technique: sit a timed practice test, find your weakest component, book early — centre availability can be limited.

Which test should I choose?+

Can I combine scores from two sittings?+

What if I narrowly miss one component?+

SECTION 08

Completing the application

Once your documents are gathered, certified and in date, the application itself is careful assembly.

1

Create your application on the Board's portal

Work through each section methodically. Read every question carefully — accuracy here saves queries later.

2

Upload your documents

Clear, high-quality colour scans, named so the Board can see at once what each file is.

3

Pay the fees

Assessment and registration fees as they apply. Keep your payment confirmation.

4

Submit

Review everything once more against the requirements. A ten-minute final check is the cheapest delay-insurance there is.

5

Respond to any requests

Expect an acknowledgement, possibly requests for further information, then the decision. Check email, including spam, and the portal. A request that sits unanswered for three weeks adds three weeks.

Put yourself in the assessor's shoes.

Could a stranger open this application and find everything they need without emailing you? If yes, you have prepared it well.

Understanding the fees

Depending on your circumstances the Board's charges can include an application or qualification-assessment fee, the registration fee, your Annual Practising Certificate and — if it applies — an examination fee. On top sit third-party costs: criminal history checks, translation, certification. The examination fee is paid in advance and forfeited if you are refused access on the day, so do not pay it until you are ready and your ID is in order.

Amounts are deliberately not printed here. The Board publishes its prescribed fees in a notice on its website and revises them; confirm the current figures on the Board's own schedule before you budget or pay. Where an Ethicare package applies to your placement, APC costs are reimbursed — ask us what your package includes before you rely on it.

SECTION 09

After you submit

Submitting is a milestone, not the finish line.

1

Acknowledgement

The Board confirms receipt. Keep your documents and login organised and watch for contact.

2

Assessment

Qualification, competence and recent practice reviewed against your scope. Timeframes vary with your background and how complete the application is.

3

Further information

Routine. A fast, complete response keeps things moving.

4

Registration decision

Confirmed, sometimes with conditions or a supervised period. The moment the move becomes real.

5

Annual Practising Certificate

Obtain your APC. It is what permits you to practise.

6

Employment and relocation

With registration, or clear registrability, you are in a strong position to secure a role. This is where [planning the move](#) takes over from registration.

— WHERE PEOPLE GET STUCK

Almost every avoidable delay traces back to one of these

Incomplete references

Warm but vague, with no clinical detail, dates or scope. The single most common cause of a follow-up request.

Poor-quality scans

Cropped, blurry or greyscale. If the assessor cannot read it, it does not count.

Missing transcripts

A certificate without the transcript that shows course content and clinical hours.

Incorrect certification

Copies certified by the wrong person or with the wrong wording — redone from scratch.

Expired Good Standing

Requested too early, lapsing before assessment, reissued.

Inconsistent names

Passport, certificates and references that differ, with no bridging document.

Gaps in employment history

Unexplained periods that raise questions about recent practice.

The fix for all of these

Prepare everything together, check it as a set, and have it read by someone else before you submit. Most of these mistakes are invisible to the person making them.

SECTION 11

The questions we are asked most

Can I apply for registration before finding a job?+

Do I need registration before interviewing?+

How long does registration usually take?+

Can I submit additional documents later?+

Can I work while my application is being assessed?+

Will I have to work under supervision?+

Will I need to sit an examination?+

What if my referee has left the organisation?+

How much does registration cost?+

Can Ethicare review my application?+

SECTION 12

The whole journey on one page

From first decision to first day at work. Each stage links to the section that explains it.

Before you start

Confirm your scope

Imaging, sonography, nuclear medicine or radiation therapy — by qualification, not job title. [Section 02](#)

Before you start

Check eligibility and your likely pathway

The Board's online eligibility check; whether supervision or an examination may apply. [Section 03](#) · [04](#)

Preparing

Gather and certify your documents

Passport, certificates, transcripts, CV, name-change documents, translations. [Section 05](#)

Preparing

Brief referees and time your Good Standing

Specific clinical references; certificates requested to arrive when you are ready. [Section 06](#)

Preparing

Evidence your English

IELTS, OET or alternative evidence, valid through to assessment. [Section 07](#)

Applying

Submit a clean application

Portal, scans, fees, final check. [Section 08](#)

Assessment

Respond quickly to anything the Board asks

The biggest factor within your control. [Section 09](#)

Decision

Registration, and your APC

Sometimes with a supervised period or conditions. Then the certificate that lets you practise.

Then

Employment and the move

Where registration hands over to [planning your move](#).

THE CHECKLIST

Forty-five items, in the order you will meet them

Tick as you go. Ticks are saved on this device only, and the list is always the current version, which a downloaded copy never is. Every document below is one the Board asks for — it describes them as a legal requirement and says provision of them is non-negotiable.

0 of 45 ticked

[Clear all ticks](#)

01 Before you start

Get oriented before you spend money or gather documents.

- ☐ **Confirm which MRTB scope you are applying for** Medical imaging, MRI, sonography, nuclear medicine or radiation therapy. MRI is a scope in its own right; mammography sits within the imaging scope.
- ☐ **Complete the Board's online eligibility check** On the Board's website, before you begin a full application.
- ☐ **Understand your likely pathway** Whether a supervised period or examination may apply — sonographers and MRI technologists more often face full assessment.
- ☐ **Check your qualification level** The New Zealand benchmark is a bachelor's-level qualification (NZQF Level 7 equivalent), assessed on content and clinical hours too.
- ☐ **Note the current fees** From the Board's live fees notice — figures change.

02 Identity and personal documents

The foundations every application needs.

- ☐ **Passport** Valid, not close to expiry; full-colour scan of the photo page, all edges visible.
- ☐ **Name-change documents** Marriage certificate, deed poll or equivalent — only if your name differs across documents.
- ☐ **Consistent name across every document** Check before you submit.

03 Qualifications and training

Evidence of what you trained to do.

- ☐ **Qualification certificates** Certified copies of your degree or diploma for the relevant scope.
- ☐ **Academic transcripts** Showing course content, modules and clinical placement hours.
- ☐ **Full course curriculum, in English** Required, not optional. If your curriculum is not in English it needs a certified translation.
- ☐ **Clinical logbook** Your record of clinical cases and procedures from training. One of the Board's listed documents and easy to leave behind when you move country — find it early.
- ☐ **Thesis or dissertation** If your qualification included one. Listed by the Board among the documents it asks for.
- ☐ **Certified copies prepared correctly** Confirm who may certify and the exact wording required.
- ☐ **Translations, if applicable** Independent, certified translation of any non-English document.

04 Experience and references

Proof your skills are current.

- ☐ **Up-to-date CV** Complete employment history, every period accounted for, no unexplained gaps.
- ☐ **Employment history compiled** Dates and hours consistent with the rest of your application.
- ☐ **Clinical experience form** The Board's own form. Complete it from your records rather than from memory — it has to agree with your CV and your employment certificates.
- ☐ **Certificate of employment from every employer in the last five years** Each one, not a summary. Chase the oldest first; former employers are the slowest part of most applications.
- ☐ **Evidence of two years' overseas employment, if it applies to you** Start and end dates, weekly hours and job title. Ask for it in that form so you are not going back a second time.
- ☐ **Suitable referees identified** People who directly supervised your clinical work — not just senior names.

- ☐ **Referees briefed** It is for New Zealand registration; share your scope and the detail the Board values.
- ☐ **Strong clinical references obtained** On letterhead, signed, dated, with scope, dates, competence and contact details.
- ☐ **References reviewed before submission** Optional — a second pair of eyes catches what the writer cannot see. We can do this.

05 Good standing and background

Confirmation you are in good professional standing.

- ☐ **Evidence of registration and a certificate of good standing from every authority** From each regulator you have been registered with, sent directly to the MRTB.
- ☐ **Two certificates of good character — one personal, one professional** A separate requirement from good standing, and the one most often missed. The Board asks for both.
- ☐ **Good Standing timing checked** They expire — time them to arrive when your application is ready, not months early.
- ☐ **Criminal history or police checks** For every country lived in for twelve months or more since age sixteen, via the Board's specified provider.
- ☐ **Checks sent directly to the Board** From the issuing authority, not forwarded by you.

06 English language

Evidence you can communicate safely in clinical practice.

- ☐ **Confirm which English pathway applies to you** Test, English-medium education evidence, or the senior-referee route.
- ☐ **Sit IELTS (Academic) or OET, if required** Meet the minimum in every component, not just overall.
- ☐ **Check test validity dates** Results expire, typically after about two years — time your test so it is still valid at assessment.

07 If you are applying for a trainee scope

Trainee Nuclear Medicine Technologist, Trainee Sonographer or Trainee MRI Technologist. Skip this stage if it does not apply.

- ☐ **Evidence of enrolment in a Board-approved training course** The course has to be one the Board approves — check before you enrol, not after.
- ☐ **The clinical site where you will train** Named on your application.
- ☐ **The name of your supervisor** Confirmed with them before you submit.

08 Submitting your application

Putting it together, cleanly.

- ☐ **Create your application on the Board's portal** Work through each section carefully and accurately.
- ☐ **Upload documents as clean colour scans** Straight, square, nothing cut off, clearly named — a portfolio, not a pile.
- ☐ **Pay the applicable fees** Keep your payment confirmation.
- ☐ **Final check against the requirements** Could a stranger find everything they need without emailing you?
- ☐ **Submit**

09 After you submit

Keep things moving to your decision.

- ☐ **Watch for Board communication** Email, including spam, and your portal — regularly.
- ☐ **Respond to any information requests promptly** Speed of response is the biggest factor within your control.

☐ **Complete any supervised practice or examination, if required** A normal part of the pathway for many international applicants.

☐ **Registration confirmed**

☐ **Apply for your Annual Practising Certificate** Required before you can practise.

Print or save as PDF

WHAT NEXT?

Registration is one part of the decision

Check my pathway Your profession, where you trained, what you hold — and the route most likely to apply to you. **How registration works in New Zealand** For every profession — the boards, the sequence, English and timelines. **Medical imaging and radiation in New Zealand** The family of professions this Board covers, and the pages for each. **Radiographer / MIT** What the work is like, indicative pay, where you could live.

Sonographer The scope with no fast-tracked pathway — and what that means for your plan. **Radiation therapist** Registration, pay and the services that recruit. **Nuclear medicine technologist** The imaging scope with the smallest workforce — and steady demand. **Plan your move** Registration, visas, family and the order to do it in — personalised, saved in your browser.

HOW WE HELP

We can read it before the Board does

We review documents and references against what the Board looks for and flag likely issues while they are still easy to fix. We support and guide you through the process; the registration decision always rests with the Medical Radiation Technologists Board.

Talk to Ethicare

The Board's own site ↗